



A SOOTHEMADE BOOK

How to Read Your Baby

Thirty cues, from the cries to the smiles.

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the parent at 3am, holding a small person whose face does not yet have a word.

You will learn the language. You are learning it already.

A note from Maya

For the first six weeks of parenting, you will spend a lot of time wondering what something means.

The cry that started fifteen seconds ago. The face he is making at the ceiling. The hand that just went into the mouth. The way she turns away from you when the room is loud. The way the eyes glaze a few minutes before sleep. The hiccups that started right after the third visitor arrived.

A baby cannot tell you what is wrong with words. So they tell you in faces, sounds, hands, and feet. The cues are subtle. You will get good at reading them. This book is the bridge between not knowing and knowing.

It is not a diagnosis tool. If something feels wrong in a way the book does not explain, the right move is the pediatrician, not the next chapter.

With care, Maya

Chapter 1: How to Read This Book

Your baby is the source language. This book is the dictionary.

For each cue in the book, you get:

- **What it looks like.** The specific behavior or sound.
- **What it often means.** The most common reading.
- **What you might try.** Suggestive, not prescriptive.
- **When it is something else.** What to watch for that might point a different direction.

A small principle before we start: every baby is different. The cues in this book are common patterns, not universal. Your baby has their own dialect. A cue that says "tired" in this book may say "hungry" for your baby. A cue that says "overstim" may say "I want to be held." The book gives you a starting place. Watching your specific baby gives you the actual answer.

The work of the first six weeks is observation. The book helps you know what to observe. Your baby teaches you what it means.

Chapter 2: The Cries

Most cries fall into a few rough categories. Once you can tell them apart, you will spend less of your day feeling like the baby is yelling at you.

A note before you start. A baby who is crying is communicating, not malfunctioning. Crying is the only big tool they have. Even when you cannot tell what it means, you can hold them while you figure it out. Holding is rarely the wrong move.

This chapter has eight cries. The first five are the most common. The last three are the harder ones, the witching hour, the cry that won't stop, the cries that send you to the pediatrician.

THE HUNGER CRY

What it looks like: Builds gradually. Often starts with mouth movements, sucking on the hand, fussiness, then escalates into a steady, rhythmic cry. Pauses if a feed seems to be coming.

What it often means: They are hungry. Even if it has only been an hour since the last feed. Newborns cluster.

What you might try: Offer a feed even if it is "too soon" by the clock. Newborns do not follow the clock. If you are bottle-feeding and just fed, try a smaller top-off rather than a full feed. If breastfeeding, offer the second side again, even if you think they finished.

When this is NOT a hunger cry: If they are also very hot, very cold, or have not had a wet diaper in many hours, hunger may not be the first answer. Check those first.

THE TIRED CRY

What it looks like: Whiny, building. Often comes with eye rubs or a slack body. They may turn their head away from stimulation. Sometimes a "shouty" cry that is the third stage of overtired.

What it often means: They missed the early tired window and are now overtired. Or they have been awake longer than their tiny body can manage.

What you might try: Reduce stimulation. Lights down, voices quiet, a room change. Swaddle if they tolerate swaddling. Hold them with their back to your chest, so they cannot see the world. Walk slowly while bouncing. The motion + dark + body warmth is often the combination.

A small reminder: Overtired babies are harder to put down than tired babies. The next time, try to catch the early window (see the sleep cues chapter).

THE PAIN CRY

What it looks like: A sharp, sudden cry that is louder than the others. Often comes in waves. The baby's body may stiffen. The face is often red.

What it often means: Something is hurting. Gas. A stuck arm or leg in a swaddle. A hair wrapped around a toe (it happens, check). A fever building. Reflux. Teething later on.

What you might try: Check the diaper. Unswaddle and check for anything pinching or stuck (especially fingers, toes, and the genitals). Burp them, even if it has been a while. Lay them on their back and slowly bicycle their legs to help release gas. Hold them against your chest, upright, for a few minutes.

Call the pediatrician if: The pain cry does not let up after fifteen or twenty minutes, the baby feels warmer than usual, or you cannot find what is hurting and your gut says something is wrong.

THE DISCOMFORT CRY

What it looks like: Whiny, intermittent. Not a full cry. More like a grumble that keeps coming back. The baby may squirm, kick, or wiggle.

What it often means: Something is uncomfortable but not painful. A wet diaper. A bunched-up onesie. Being too hot or too cold. Needing to burp. Reflux.

What you might try: Diaper check. Strip them down and see if a tag, seam, or wet patch was the issue. Adjust the room temperature. Newborns are often most comfortable in a layer or two less than you would guess. Burp them. Try a few different positions. Hold them upright for ten minutes.

THE OVERSTIM CRY

What it looks like: Often happens later in the day. The baby was fine, then suddenly is not fine. The cry may sound "too big" for what is happening. Eyes squeeze shut. They turn away from light, noise, faces.

What it often means: Too much input. A long visit. A bright store. A noisy meal. The fifth person who wanted to hold them. The world is too loud and they need it to stop.

What you might try: Go to the quietest room in the house. Dim the lights. Hold them against your chest with their face turned away from the room. Keep your voice low. Hum or shush. Words can be too much. Do not pass them to a new person. They need stability, not novelty.

A note: The overstim cry often comes around 5 to 8pm. This pattern is sometimes called "the witching hour." It is not a sign anything is wrong with your baby. It is a sign their tiny nervous system is reaching the end of its day.

THE "I DON'T KNOW WHAT I WANT" CRY

What it looks like: Inconsistent. Stops when you pick up, then starts again. Stops when you offer a feed, then starts again. Stops when you put down, then starts again.

What it often means: They are not sure either. They may be tired and hungry and overstimulated at the same time. They are also tiny and their needs change minute to minute.

What you might try: Run the basic checklist: diaper, feed, burp, temperature, swaddle, motion. If nothing seems to be the answer, hold them and walk. Sometimes the only answer is contact + movement + time. It is okay to put them down in a safe spot for five minutes if you are reaching the end of your patience. A safe spot is a crib or bassinet on the back, with nothing else in it.

THE WITCHING-HOUR CRY

What it looks like: A long, sustained cry, often from 5 to 8pm. The baby is inconsolable for stretches. You try everything. Nothing works for long.

What it often means: Most newborns have a witching hour. It is part of how their nervous system processes the day. It usually fades by 8 to 12 weeks.

What you might try: Settle in. Plan for this hour. Have your dinner earlier, or order in. Walk with them in a carrier, outside if you can. Outside often helps. Hand off to a partner for a stretch so you can step away. Lower the lights, lower the noise. The day was loud. A warm bath sometimes helps. Not always. Sometimes makes it worse. Try it once and see.

Do not try: Adding more stimulation in an attempt to "tire them out." The witching hour is overstim, not underrest.

THE CRY THAT WON'T STOP

What it looks like: None of the usual things work. The cry has been going for thirty minutes or longer. You have tried feed, diaper, burp, swaddle, walk, dark room. Nothing.

What it often means: Sometimes nothing in particular. The baby is having a hard hour. Sometimes more. Sometimes a sign of pain you cannot see.

What you might try: Put the baby down somewhere safe. Step out of the room for a minute. Take a breath. Drink water. Call your partner, your mother, a friend. Have someone come hold the baby while you take ten minutes.

Call the pediatrician if: The cry is high-pitched and different from any cry you've heard before, the baby has been crying inconsolably for more than two hours, the baby has a fever, the baby is hard to wake or feels unusually limp, or your gut is telling you something is wrong. Trust the gut.

Chapter 3: The Sleep Cues

The trick to sleep with a newborn is catching the cues early. After the cues, the window has closed and you are in the tired-cry zone. Watch for these.

There are six cues in this chapter. They tend to appear in roughly this order, from earliest signal to "you missed the window." The earlier you catch them, the easier the sleep transition.

THE FIRST YAWN

What it looks like: A small yawn. Maybe the baby has been awake for 45 to 90 minutes. The eyes are still open. Energy is still up.

What it often means: This is the start of the tired window. If you can begin the sleep routine now, the rest is much easier.

What you might try: Start dimming the lights. Move toward the bassinet or the carrier. Reduce stimulation.

Do not wait for: The second yawn. By the third yawn, the window is closing.

THE MIDDLE-DISTANCE STARE

What it looks like: The baby is looking at something, but not focused. Eyes are slightly glazed. May seem mesmerized.

What it often means: The brain is starting to disengage from the world. Sleep is coming.

What you might try: Don't try to engage them. Don't talk to them. Don't bring them to a new spot to "say goodnight." Start the sleep environment.

THE SLACK ARMS

What it looks like: The baby was holding their hands up, or their arms were active. Now the arms are loose. The body is melting.

What it often means: They are ready. The window is open. Move now.

What you might try: Swaddle if they tolerate it. Move to the dark, quiet space. Put them down or settle them on you.

THE DISENGAGED EYE

What it looks like: The baby turns their head away from your face, from the toy, from the light. Their eyes do not come back.

What it often means: They are done. They cannot take any more input.

What you might try: Stop the activity. Whatever you were doing, stop. Move to the sleep space. Reduce light and sound to almost zero.

THE EYE RUB

What it looks like: The baby brings a hand to the eye and rubs, or pushes the face into your shoulder.

What it often means: Late tired. Window is about to close.

What you might try: Move quickly to settle. Skip the bath, skip the changing, skip the photo. Get them into the sleep position.

"I MISSED THE WINDOW"

What it looks like: Suddenly the baby is fussy. Crying. Shouty. Cannot be settled in the usual way. You think, "wait, weren't they fine ten minutes ago?"

What it often means: You missed the early sleep cues. They are now overtired. Sleep will be harder.

What you might try: Don't beat yourself up. Every parent misses the window. The next nap is a new chance. Use the strongest settle tool you have. Carrier walk outside. Long bounce. Deep shush. Lower expectations for this nap. It may be short and hard. Next time, watch for the first yawn earlier.

Chapter 4: The Hunger Cues

The official advice is to feed before the cry. The crying baby is a late-stage hungry baby. These are the earlier signals.

There are six cues in this chapter, from "earliest possible signal" to "you waited too long."

ROOTING

What it looks like: The baby turns their head toward whatever touches their cheek. The mouth opens. They look for something to latch onto.

What it often means: Hungry. Often before they will cry about it.

What you might try: Offer the feed. If breastfeeding, offer the breast. If bottle-feeding, the bottle. If they were just fed, they might want a small top-off, or might want to comfort-suck. A pacifier is okay if you are using them.

HAND-TO-MOUTH

What it looks like: The baby brings a fist or hand to their mouth and sucks on it. May also chew on a sleeve.

What it often means: Hungry, or about to be hungry. Sometimes also self-soothing. Context matters.

What you might try: If it has been more than 1.5 hours since the last feed, try offering food. If it has been less, they may be self-soothing rather than hungry. Offer a pacifier if you use them.

SMACKING LIPS

What it looks like: The baby is awake-ish. Lips are moving. Tongue may be sticking out a little. They look like they are tasting something.

What it often means: Hungry. Often a very early signal.

What you might try: Offer the feed. They are early enough that the feed will be calm.

THE EARLY RESTLESS

What it looks like: The baby was sleeping. Now they are starting to wiggle. Eyes are still closed. Body is squirming.

What it often means: They are waking, and they will be hungry when they fully wake.

What you might try: Get ready. Warm a bottle if needed. Get to a comfortable feeding spot. They may settle and go back to sleep. Or they may fully wake. Either way, you are prepared.

"IF YOU WAIT, THEY CRY"

What it looks like: A few minutes of rooting and hand-to-mouth, escalating to a fuss, escalating to a cry.

What it often means: You waited too long. They moved from hungry to angry.

What you might try: Feed them. Even though they are crying, the feed is still the answer. Some babies are too dysregulated to latch when crying. Calm them first, then feed. Next time, try to catch the earlier cue (rooting, hand-to-mouth, smacking lips).

THE CLUSTER NIGHT

What it looks like: They want to feed every 30 to 90 minutes for a whole evening. Or all night. Multiple nights in a row, sometimes.

What it often means: A growth spurt, a developmental leap, or just a cluster pattern. Not a sign anything is wrong.

What you might try: Settle in. Hydrate. Snacks within reach. Phone charged. If breastfeeding, the cluster is signaling the body to make more milk for the next phase. If bottle-feeding, smaller bottles offered more often is fine. Common cluster ages: day 3, week 2 to 3, week 6, three months. They pass.

Chapter 5: The Overstimulation Cues

Newborns reach their stimulation limit faster than you think. The signs are subtle. Catching them early is the difference between a calm evening and a hard one.

This chapter has six cues. The first three are the body language of "too much." The last three are time-of-day patterns that almost every newborn goes through.

THE HEAD-TURN-AWAY

What it looks like: You are smiling at them, talking to them, or holding them in a way they can see your face. They turn their head away.

What it often means: Too much. They need a break.

What you might try: Stop the engagement. Hold them so they cannot see your face. Their face toward your shoulder, or facing away from a stimulating room. Lower your voice.

THE ARCHED BACK

What it looks like: The baby pushes their back away from you. May extend their legs. Sometimes mistaken for "they don't like me" or "they don't want to be held."

What it often means: Overstimulation, often. Sometimes reflux discomfort. Sometimes wanting to be in a different position.

What you might try: Move to a quieter space. Try a different hold. Upright, against your chest, with their head on your

shoulder. If consistent, especially around feeds, mention to the pediatrician at the next visit.

THE FROZEN STARE

What it looks like: The baby stops moving. Eyes are open and staring, but not engaging. They are not asleep, not awake-engaged. They look "checked out."

What it often means: Their nervous system has tried to manage the input and has gone into a freeze response. This is the body's way of saying "too much."

What you might try: Reduce input immediately. Quiet room. Low light. Hold them. Do not try to re-engage them by smiling or talking. Let them come back. They will. It may take five or ten minutes.

THE HICCUPS IN A CROWD

What it looks like: The baby starts hiccuping during a busy event. Visitors, a restaurant, a holiday gathering.

What it often means: Hiccups are often a stress response in newborns. The body is reacting to too much.

What you might try: Leave the room. A quiet spot for ten minutes. A feed sometimes helps hiccups, sometimes does not. Hold them upright.

THE 6PM HOUR

What it looks like: The baby was fine all day. By 5 or 6pm, they are fussy, crying, hard to soothe.

What it often means: The accumulated stim of the day. The end-of-day fatigue. The "witching hour" overlap.

What you might try: Plan for this hour. Don't schedule anything else. Lower the household stim. Dim lights. Quiet voices. Skip

the news. Carrier walk outside. Hand off to the partner if you can.

"WE DID TOO MUCH TODAY"

What it looks like: You took the baby to two appointments and a grocery store. By the evening, they cry through everything. Bedtime is hard.

What it often means: The day was too full. Newborns cannot tolerate a packed day.

What you might try: Tomorrow, do one outing maximum. Build in recovery time after any outing. Watch the cues at the second outing. If they were already overstim, the third stop was too many.

A reminder: Newborn capacity is much smaller than your capacity. The errands list is yours, not theirs.



Chapter 6: The Connection Cues

These are not crying signals. They are the small moments of connection that are easy to miss when you are exhausted. The book would not be complete without them.

There are four cues in this chapter. They are short on purpose. The moments themselves are short. The point is to notice them.

THE EARLY EYE CONTACT

What it looks like: The baby's eyes find yours and hold for a moment. Often in the first few weeks, longer than you would expect for a newborn.

What it often means: They are practicing connection. They are learning your face.

What you might try: Hold their gaze. Talk to them softly. They are listening. This is one of the moments worth pausing for. The dishes can wait.

THE FIRST SOCIAL SMILE PRECURSOR

What it looks like: Around 4 to 8 weeks, you may see a small smile that seems to be in response to you, not gas. The eyes light up too.

What it often means: They are starting to socially engage.

What you might try: Smile back. Talk softly. Wait. They may smile again. This is also worth pausing for. Real social smiles are a milestone.

Note: Earlier smiles (the first few weeks) are often reflexive, not social. That is fine. Real social smiles arrive later.

THE HAND ON YOUR CHEST

What it looks like: The baby is on your chest, and their hand finds your skin or your shirt and rests there.

What it often means: They have found you. They are settled.

What you might try: Stay still for a minute. Notice that you are someone's whole world right now.

"I JUST WANT YOU"

What it looks like: You hand them to someone else and they fuss. You take them back and they settle.

What it often means: They have learned who you are. They prefer you. This is attachment forming, in real time.

What you might try: It is okay to take them back. The "I should be able to hand them off" pressure is not yours to carry. They are wired to want you. That is not a problem.

Chapter 7: When the Cue Isn't Fixing It

The book describes common cues. Your baby is not common. They are themselves.

If you are doing what the book suggests and the cue is not resolving, the book has done its job by getting you started. The next step is one of these:

Trust the next instinct. The hold may need to be different. The room may need to be darker. The next feed may need to be sooner. The book is a starting place. Your hands have learned more than you have words for.

Hand off. Let your partner or another adult try. A fresh person is sometimes the answer. Babies often respond differently to different bodies, and your own depletion may be part of what is not working.

Walk outside, in a carrier. Outside almost always changes things. Fresh air, motion, dappled light, a different soundscape. Even for ten minutes. Even if it is cold.

Lower the stimulation more than you think is necessary. Pitch black, no sound, body heat, slow rocking. New parents often underestimate how dark and quiet a baby needs the environment to be.

If nothing is working and a meaningful amount of time has passed (say, thirty to forty-five minutes of trying), it may be a time to call the pediatrician. Not because the cue is dangerous. Because your gut is telling you something. Trust the gut.

Chapter 8: When to Call the Pediatrician

The book is for the everyday confusion. This chapter is for the moments that are not the everyday confusion.

Call the pediatrician (or 911 if appropriate) if any of these are true:

- The baby has a fever. For a newborn under 3 months, any fever is a call-the-provider thing. Your provider will tell you their threshold.
- The baby is breathing fast, shallow, or noisily in a way that is new.
- The baby is hard to wake, or feels unusually limp.
- The baby is not feeding, or not keeping feeds down, for more than two feed cycles.
- The baby has not had a wet diaper in many hours.
- The baby has a rash that is spreading or that bothers you.
- The cry is high-pitched and different from any cry you have heard before.
- The baby has been inconsolable for more than two hours despite trying everything.
- Your gut is telling you something is wrong.

The gut is the most important one. You know this baby better than the internet. If the gut is saying call, call.

A practical note: keep the numbers somewhere you can find them at 3am. Your phone. A fridge magnet. A note in your

hospital-bag-leftover bag. Not "you will remember where you saved it." Saved.

- Your pediatrician's main number
 - The after-hours line
 - The nearest ER (drive-time noted)
 - The number for the on-call advice nurse, if you have one
 - Poison Control: 1-800-222-1222 (US)
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Chapter 9: A Final Note

The first six weeks are the densest learning period of your life.

By week eight, you will read your baby's face the way you read a friend's. You will know the difference between the hunger cry and the tired cry without thinking. You will know which fuss is "I need to be put down" and which is "I need to be held closer."

The book is the bridge. After the bridge, you will not need it. Many parents do not need it past the third month.

You are reading a language that is being invented in real time, by a tiny person, and you are getting better at it every day.

With care, Maya



About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents and the people taking care of them.

The printable companion to this book is the **Newborn Cue Decoder Cards** at notes.soothemade.com, sized to print on cardstock and slip into the diaper bag.

Also from Soothemade

- **30 Scripts for the Postpartum Visitor**, for the doorbell and the unsolicited advice while you are also reading your baby.
 - **The Field Guide to Asking for Help**, for the moments when you need a person more than a script.
 - **The Newborn Sleep Log** (printable), if you want to track patterns over time.
 - **The Breastfeeding Troubleshooting Guide** (printable), if feeding is the cue you are most worried about.
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Crisis lines

If you are struggling beyond the everyday confusion, the lines below are open.

United States

- 988, Suicide and Crisis Lifeline
- 1-833-852-6262, Postpartum Support International
- 1-833-943-5746, Maternal Mental Health Hotline
- 1-800-222-1222, Poison Control (US)

United Kingdom

- 116 123, Samaritans

Canada

- 9-8-8, Suicide Crisis Helpline

Australia

- 1300 726 306, PANDA

This book is for planning and personal use only. It is not medical advice. For specific concerns about feeding, sleep, breathing, or behavior, please call your pediatric provider.

A Soothemade book. Made slowly, in plain language.